

ICSR Case Processing Steps

Full Tutorial Summary: End-to-End Individual Case Safety Report Lifecycle

Overview: An Individual Case Safety Report (ICSR) documents adverse events, product quality complaints, or special situation scenarios linked to a medicinal product. Systematic end-to-end processing ensures compliance with global regulatory timelines and continuous drug safety surveillance.

1. FOUR CORE CRITERIA FOR CASE VALIDITY

Before initiating processing in a safety database (e.g., Argus, ARISg), an ICSR must meet four minimum validity criteria:

1. Identifiable Patient Patient initials, age, date of birth, gender, or unique identification number.	2. Identifiable Reporter HCP, consumer, pharmacist, or study coordinator contactable for confirmation.
3. Suspect Medicinal Product Trade name, active substance, formulation, dose, or batch/lot details.	4. Adverse Event (AE / ADR) Signs, symptoms, lab findings, or clinical diagnosis occurring post-exposure.

2. KEY STAGES OF THE ICSR PROCESSING LIFECYCLE

The standard operational workflow following the receipt of an adverse event report consists of seven primary stages:

STAGE	PRIMARY ACTION	KEY OPERATIONAL FOCUS
1. Triage & Intake	Verification of validity criteria & Day 0 assignment.	Determines initial seriousness and priority, initiating the regulatory clock.
2. Duplicate Search	Checking the safety database for existing entries.	Prevents duplicate case creation by matching patient, reporter, and event dates.
3. Data Entry	Capturing case details into structured fields.	Enters medical history, concomitant drugs, dosages, start/stop dates, and lab results.
4. Medical Coding	Standardized terminology assignment.	Codes adverse events using MedDRA and drugs using WHO Drug Dictionary.

3. REMAINING PROCESSING STAGES & MEDICAL EVALUATION

STAGE	PRIMARY ACTION	KEY OPERATIONAL FOCUS
5. Medical Assessment	Causality, Seriousness & Expectedness review.	Evaluates temporal relationship, dechallenge/rechallenge, RSI alignment, and narrative synthesis.
6. Quality Review (QC)	Audit of data accuracy and coding consistency.	Verifies source documents against entered fields prior to submission lock.
7. Reporting & Follow-Up	Regulatory submission and ongoing tracking.	Submits expedited reports (7/15-day rules) and requests missing clinical information.

4. ESSENTIAL SAFETY ASSESSMENTS

During the medical evaluation phase, three critical determinations dictate regulatory handling:

Seriousness Evaluation Determines if the event meets SAE criteria (Death, Life-threatening, Hospitalization, Disability, Congenital Anomaly, Medically Important).	Expectedness / Labeling Cross-references the event against Reference Safety Information (SmPC, IB, CCSI) to flag unexpected reactions.
Causality Assessment Applies standard scales (WHO-UMC or Naranjo) to establish relationship likelihood (Certain, Probable, Possible, Unlikely).	Case Narrative Creation Drafts a chronological summary of patient demographics, clinical course, intervention, and final outcome.

5. REPORTING TIMELINES

- **7-Day Expedited Reporting:** Mandatory for fatal or life-threatening Suspected Unexpected Serious Adverse Reactions (SUSARs). Initial notification required within 7 calendar days, followed by a complete report within +8 additional days.
- **15-Day Expedited Reporting:** Applies to all other serious, unexpected adverse drug reactions (ADRs) post-marketing or from clinical trials.
- **Non-Expedited Cases:** Non-serious or expected events aggregated into periodic safety reports (PSUR / PBRER / DSUR).

Core Takeaway: Compliance with Day 0 clock management, rigorous duplicate checking, and standardized MedDRA coding form the foundation of compliant ICSR processing and patient safety surveillance.

